INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cisplatin + Etoposide

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00280
Version 7

> Cisplatin is given on Day 1. Etoposide is given on Days 1–3. A cycle lasts 21 days (3 weeks); the team checks you before the next cycle.

Treatment stages

01 **Day 1 platinum and etoposide; Days 2–3 etoposide**
Three treatment days, then recovery

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

> Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Cisplatin + Etoposide
National Cancer Control Programme (NCCP) 00280

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call urgently for fever or shaking chills, sudden illness, new or worsening breathlessness, chest pain, uncontrolled bleeding, severe diarrhoea, vomiting despite medicines, inability to drink, very little urine, or a spreading/blistering rash. Tell staff immediately about wheeze, swelling, dizziness or faintness during an infusion. Report hearing changes, ringing in the ears, numbness, weakness or difficulty walking promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

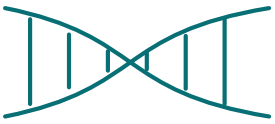
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your two chemotherapy medicines

Cisplatin + Etoposide is chemotherapy used for extensive-stage small-cell lung cancer. It aims to control the cancer and its symptoms. Ask your oncology team what benefit is realistic for you and how this option compares with other suitable treatments.

TOPOISOMERASE II CHEMOTHERAPY

Etoposide



Disrupts deoxyribonucleic acid (the cell's genetic material) (DNA) repair during cell division

Etoposide interferes with topoisomerase II, an enzyme that helps manage DNA during copying. DNA breaks can remain unrepaired and cancer cells may stop dividing or die. Healthy cells are also affected; the infusion or prescribed capsules deliver the same medicine by different routes.

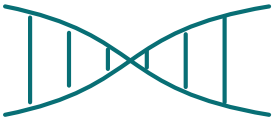
Topoisomerase II disrupted → DNA breaks persist → Cell division interrupted

What to watch for

Report fever, bleeding, mouth sores or diarrhoea. Tell the nurse about faintness, rash or breathing difficulty during an infusion. If capsules are prescribed, use the exact pharmacy instructions and contact the team about a missed or vomited dose.

PLATINUM CHEMOTHERAPY

Cisplatin



Forms links that damage DNA

Cisplatin forms links in DNA that interfere with copying and repair. This helps stop cancer cells dividing; normal tissues can also be affected. Kidney protection and assessment of hearing and nerves are essential during cisplatin treatment.

Platinum enters cells → DNA links form → DNA copying disrupted

What to watch for

Report changes in urine, hearing, ringing in the ears, numbness or daily function. Tell the team about infection, unusual bruising or bleeding and any symptoms during the infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Evidence and your own treatment decision

The exact medicines, dose, population and treatment era matter.

Platinum and etoposide are established chemotherapy components for extensive-stage small-cell lung cancer (SCLC). A pooled comparison found no clear survival difference between carboplatin-based and cisplatin-based treatment, but their side effects differed. The elderly/poor-risk JCOG9702 trial used etoposide 80 mg/m² and split cisplatin, so its survival figures cannot be assigned directly to this 100 mg/m² etoposide schedule. These older chemotherapy comparisons do not rank current immunotherapy combinations or predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

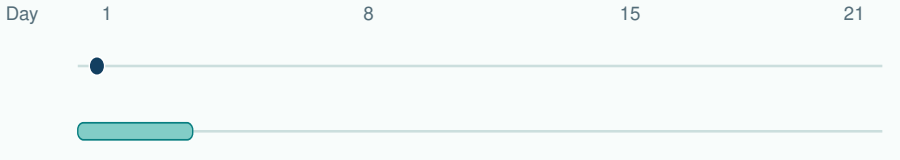
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Three treatment days in a 21-day cycle

Cisplatin + Etoposide

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE TREATMENT DAYS, THEN RECOVERY

Day 1 platinum and etoposide; Days 2–3 etoposide

D1 D2–D3 D4–D5 D6–D21

Cisplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.

The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.

BASELINE → TREATMENT → FOLLOW-UP

Checks through the whole course

The columns show the order of care, not equal time intervals. Extra tests depend on symptoms, results and the consultant's plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00280 v7 · official regimen · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance · Manufacturer cisplatin product information

THE WHOLE TREATMENT JOURNEY

Cisplatin + Etoposide: treatment and checks

Extensive-stage small-cell lung cancer · chemotherapy

Cisplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Days 1–3 of each cycle	Between visits	Course / response review	After chemotherapy
Chemotherapy course NCCP 00280	Agree indication, route and course	Day 1 Cisplatin + etoposide; etoposide continues Days 2–3	Recovery and symptom monitoring	The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.	Follow-up or a separately prescribed next plan
 **Blood counts** NCCP 00280	Baseline blood count	Review results before treatment	Further counts if symptoms or clinical plan require	Review infection, bleeding and previous toxicity	Follow unresolved low counts
Kidneys, liver and fluids NCCP 00280	Renal/liver profile; verify clearance and dose suitability	Renal/liver tests before each cycle; hydration/ electrolytes with cisplatin	Repeat sooner after dehydration or illness	Review organ function before continuation or modification	Follow kidney/ electrolyte/ liver abnormalities
Hearing and nerves NCCP 00280	Record baseline symptoms; audiology if indicated	Review symptoms before each cycle	Report tinnitus, hearing loss, numbness or loss of function	Balance benefit and lasting toxicity	Review persistent changes
Prevention and route NCCP 00280 · ANTIEMETIC	Confirm infusion or capsule plan and a written multi-day anti-sickness schedule	Check support before each treatment day; pharmacy confirms oral doses	Follow support through two days after last systemic anticancer therapy (SACT); do not extend medicines yourself	Review interactions and support after any dose/route change	Confirm which support medicines stop or continue
 **Disease assessment** NCCP 00280	Confirm extensive-stage SCLC and individual treatment aim	Assess symptoms and fitness	Extra assessment for new concerns	Scans/tests as directed by consultant; no universal scan interval	Agree follow-up or a separate next treatment plan

What each check is for

1

Before first treatment

Before dose authorisation

Confirm suitability and route.

Ask / act: Confirm the complete plan with the consultant and pharmacy.

2

Across Days 1–3

Every treatment day

The active medicines and route change within each cycle.

Ask / act: Confirm dose, route and today's supportive medicines.

3



After the last treatment day

Two days after multi-day SACT

Prevention extends beyond the final dose.

Ask / act: Provide an individual written schedule and rescue instructions.

4



Course and response review

During and at the end of planned chemotherapy

Benefit and toxicity guide the next step.

Ask / act: Confirm completion, persistent-toxicity follow-up and any separate next regimen.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before each visit

Confirm transport for Days 1–3, blood-test arrangements, prescriptions and the 24-hour contact details. Tell the team if you are ill before travelling.

STEP 2

Time in the unit

Etoposide is infused over about one hour before cisplatin, which is infused over about one hour on Day 1. Extra intravenous (IV) fluid and electrolytes before/after cisplatin make this a longer visit. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation.

STEP 3

Infusion access

Treatment may use a cannula or an existing suitable central line. Ask for line-care instructions if you have a line. There is no routine take-home infusion pump.

STEP 4

Capsules and home medicines

If prescribed, confirm your etoposide capsule doses, empty-stomach timing, safe handling and what to do if a dose is missed or vomited. Keep prevention and rescue medicines clearly labelled.

STEP 5

Support between visits

Arrange practical help if needed and keep symptom notes. Bring questions and a current medication list to review.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cisplatin	 **Day 1** Cisplatin on Day 1 after clinical review
Etoposide	 **Days 1–3** Infusion Days 1–3, or prescribed Day 1 infusion / Days 2–3 capsules
Prevention medicines	 **Days 1–5** Follow the individually reconciled multi-day plan
Report symptoms	 **Days 1–21** Symptoms may occur at any time; this bar does not measure risk
Next-cycle review	 **Days 20–21** Tests and review before deciding the next cycle; appointments may differ

Before the first dose: Confirm the diagnosis, treatment aim, blood tests, kidney/liver function and selected etoposide route.

During the three treatment days: Day 1 includes both medicines; Days 2–3 continue etoposide. A split cisplatin prescription, if chosen, needs a separate checked day-by-day plan.

Support after the last dose: The multi-day sickness-prevention plan needs to account for two days after the last SACT day. Take only the written prescribed course.

Across and after the course: Counts, kidneys, hearing and nerves need review throughout treatment. Some problems can persist after the final dose.

> **Why these bands stay broad:** This shows one reference cycle and the order of care. It does not predict a symptom-free day or when blood counts will fall. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00280 v7 · official regimen ↗ · NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance ↗

There are no guaranteed "safe days". Follow your unit's temperature threshold and emergency instructions. Sudden illness or shaking chills needs urgent advice even without a high temperature.

Between-treatment notes

Day 1: Day 1 in the unit

Etoposide is infused over about one hour before cisplatin, which is infused over about one hour on Day 1. Extra intravenous (IV) fluid and electrolytes before/after cisplatin make this a longer visit. Etoposide alone is given on Days 2 and 3 unless the prescribed capsule alternative is selected. The whole visit also includes review, preparation, prevention medicines and observation. Tell staff immediately about rash, swelling, wheeze, faintness or pain near the drip.

Day 2–3: Etoposide continues

Return for etoposide infusions unless the team prescribed the oral alternative. If you have capsules, follow the twice-daily pharmacy label and empty-stomach instructions. Confirm today's anti-sickness medicines. Contact the team about a missed or vomited capsule dose.

Day 4–5: After the last treatment day

Follow the written prevention and rescue plan. Multi-day antiemetic cover extends to two days after the final treatment day; the exact medicines are chosen by your team. Call for sickness despite medicines, inability to drink or reduced urine.

Day 6–20: Recovery and symptom monitoring

Fatigue, low counts, infection, mouth or bowel symptoms can occur between visits. Hearing and nerve symptoms may build across cycles. Use the urgent contact plan at any time; do not wait for the next appointment.

Day 21: Review before the next cycle

The team reviews blood counts, kidney/liver function and symptoms before deciding to continue, delay or change the plan. Bring your medicine list and symptom notes; confirm the remaining course and next review.

Support around the medicines

1 Preventing sickness across all treatment days

Take the written anti-sickness plan even if you feel well. The plan must cover the risk of the medicines on each of Days 1–3 and the two days after the last treatment. Ask the pharmacist to show which medicines are scheduled and which are rescue medicines.

2 One clear steroid plan

The anti-sickness plan may include dexamethasone. Follow the prescribed day-by-day instructions; do not add an extra steroid course or extend it yourself. Tell the team about diabetes, poor sleep or mood changes.

3 Fluids and kidney protection

Cisplatin needs IV fluid before and after treatment and may need magnesium/potassium replacement. Your team adapts this for your kidneys, heart and fluid balance. Ask for a personal drinking plan and call if you cannot drink or pass much urine.

4 If etoposide capsules are prescribed

The source alternative uses capsules on an empty stomach twice daily on Days 2 and 3. Your pharmacy label gives your individual capsule dose. Do not substitute capsules for an infusion yourself, crush/open capsules, double a missed dose or repeat a vomited dose without advice. Obtain safe handling and storage instructions.

5 Medicines, mouth care and infection advice

Check prescriptions, painkillers, supplements and vaccines with the oncology team. Some medicines affect kidney/hearing toxicity or etoposide exposure. Follow mouth-care advice and keep the unit's urgent contact instructions available.

Your treatment and its aim

Confirm the full plan before starting.

The treatment covered here

This summary is for chemotherapy for extensive-stage small-cell lung cancer. Treatment that also includes immunotherapy or radiotherapy needs its own complete plan.

Why two medicines?

The platinum drug damages DNA and etoposide interferes with DNA repair. Using them together attacks cell growth in different ways. Healthy cells can also be affected.

Before treatment

Preparation helps the team choose a suitable plan.

Tests and fitness

Blood counts, kidney and liver tests are checked before treatment. Tell the team about existing nerve/hearing problems, other medicines and any recent infection.

Route and practical arrangements

Confirm whether all three etoposide doses are infusions or whether you have capsules on Days 2 and 3. Do not substitute one route yourself.

Fertility, contraception and pregnancy

Discuss fertility preservation and contraception before treatment. Pregnancy and breastfeeding are exclusions. Ask for medicine-specific advice and the required contraception period.

Your visits and home plan

Three treatment days form one cycle.

Days 2 and 3

Continue etoposide by the prescribed route, with risk-appropriate anti-sickness prevention each treatment day.

The following days

Continue only the written prevention schedule and monitor symptoms. The two days after the final treatment day are part of the multi-day antiemetic review.

Effects to discuss and monitor

Report problems early; they are not a measure of benefit.

Blood counts and infection

Both medicines may lower blood counts. Infection can become serious quickly; follow the urgent advice plan for fever, chills or sudden illness.

Hearing, nerves and kidneys

Platinum medicines may affect hearing, nerves and kidneys, and some changes can last. Tell the team promptly about tinnitus, hearing change, numbness or reduced urine.

Mouth, bowel, hair and fatigue

Mouth soreness, nausea, bowel changes, hair loss and tiredness can affect daily life. Ask for help with symptoms; call urgently if you cannot drink or symptoms are severe.

Later effects

Etoposide can be associated with a later blood cancer. Discuss serious and lasting risks together with likely benefit and alternatives.

When to seek urgent help

Use your unit’s 24-hour number and emergency instructions.

When calling

Give your regimen, last treatment day and symptoms. Explain whether you can drink and pass urine. Do not wait for the next appointment or for a high temperature if you feel very unwell.

Review and the next plan

Completion and follow-up depend on the treatment aim and response.

Course review

Disease assessment

Scans and other assessments are arranged by the consultant. Symptoms alone do not show whether treatment is working.

After the last dose

Review ongoing kidney, hearing, nerve and blood-count problems. Confirm which support medicines finish and agree follow-up. Any next treatment, including immunotherapy or radiotherapy, needs a separate plan.

Questions worth asking

Why are there three treatment days?

Etoposide is planned on three consecutive days. Usually you attend for infusions on all three. The alternative capsule plan is only used when prescribed and clearly labelled by your team.

Are etoposide capsules the same dose as the drip?

No. The source oral alternative has a different dose and twice-daily schedule. Your prescription and pharmacy label specify your individual dose and capsule strengths. Do not convert it yourself.

Will I have radiotherapy or immunotherapy too?

This guide covers Cisplatin + Etoposide chemotherapy for extensive-stage small-cell lung cancer. Immunotherapy combinations and limited-stage treatment with radiotherapy use separate regimens. If an additional medicine or radiotherapy is prescribed, obtain its own plan and consent information.

How long does the course last?

The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response. The passport pages are reusable records, not a prescription for a fixed number of visits.

Do more side effects mean treatment is working?

No. Side effects do not reliably show whether the cancer is responding. Report them so they can be assessed and treated; your team reviews benefit with the agreed clinical and scan plan.

Why might treatment be delayed or changed?

Blood counts, kidney/liver function or symptoms may need time to recover. A dose may change to reduce lasting harm. Ask which medicine changes, why, and when it will be reviewed.

What should I say when I call?

Give the regimen name, last treatment day, symptoms and when they started. Explain whether you can drink, pass urine and manage usual activities. Give your temperature if available, but do not delay calling to collect information.

Can this guide decide whether treatment is safe?

No. Treatment decisions need your clinical results, symptoms, previous toxicity and prescriber/pharmacy review. This guide helps you understand and discuss the plan.

What should I be able to explain before I leave?

Ask your nurse or doctor to check that you can explain which medicines you are receiving, the treatment days, which medicines to take at home, and which number to ring urgently. Ask for the plan in writing if any part is unclear.

> **Try explaining the plan back:** Can you describe which days you have treatment, whether you have etoposide capsules, how you take prevention medicines and which number to call at any hour?

Which treatment this guide covers This guide covers Cisplatin + Etoposide chemotherapy for extensive-stage small-cell lung cancer. Immunotherapy combinations and limited-stage treatment with radiotherapy use separate regimens. If an additional medicine or radiotherapy is prescribed, obtain its own plan and consent information.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cisplatin + Etoposide · standard three-day course

Active medicines: cisplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	NCIS high-risk reference course for Day 1 platinum Reconcile the full multi-day course; the special extension after five-day cisplatin is not automatically applied to this three-day etoposide course.
Ondansetron	16 mg orally Day 1 (high-risk reference)	Before Day 1 treatment NCIS low-risk etoposide-day reference is ondansetron 8 mg orally once daily or a corticosteroid. Days 2–3 prevention must be reconciled with the existing dexamethasone course; do not duplicate agents automatically. Adjust for QT/ individual risk.

Medicine	Dose / route	When
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4 (high-risk reference)	A component of the individual Days 1–5 plan These are the NCIS high-risk row doses. Pharmacy must reconcile overlapping delayed-platinum and etoposide-day prevention, and specify the Day 5 plan; do not invent a uniform steroid extension.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days / rescue instructions set by prescriber High-risk NCIS row has no end day or PRN qualifier. The low-risk row uses PRN. Specify role/duration and check suitability; avoid unreviewed duplication.
Olanzapine	5 mg orally Day 1 and daily Days 2–4 (high-risk reference)	Part of the reviewed multi-day antiemetic plan Review sedation and metabolic effects. Concurrent metoclopramide increases extrapyramidal risk. Consultant may consider 10 mg; do not alter automatically.

Additional phase-specific support

- NCIS V8 p 5: provide antiemetics appropriate to the risk of systemic anticancer therapy (SACT) on each treatment day and for two days after; for standard Days 1–3 treatment, review coverage through Day 5.
- Day 1 high-risk platinum and Days 2–3 low-risk etoposide are different exposures. Table 1 alone does not give one universal three-day platinum/etoposide prescription.
- Do not invent extra steroid days or extend aprepitant using the separate five-day cisplatin exception.
- Confirm one written steroid/antiemetic course, QT risk, interactions and rescue medicines.
- intravenous (IV) and oral etoposide alternatives require matching pharmacy and patient instructions.
- Source suggested hydration: etoposide in 1 L 0.9% NaCl over 1 hour may serve as prehydration; after cisplatin,10 mmolMgSO4+20 mmolKCl in 1 L 0.9% NaCl over 2 hours, adapted to local policy and patient fluid/electrolyte status.
- Mannitol may be used per local policy; routine furosemide is not recommended without fluid overload.
- If cisplatin is fractionated over three days, each day's risk, hydration and antiemetic course need separate consultant/pharmacy reconciliation.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Your anatomical side-effect map

Cisplatin + Etoposide

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Ears / hearing

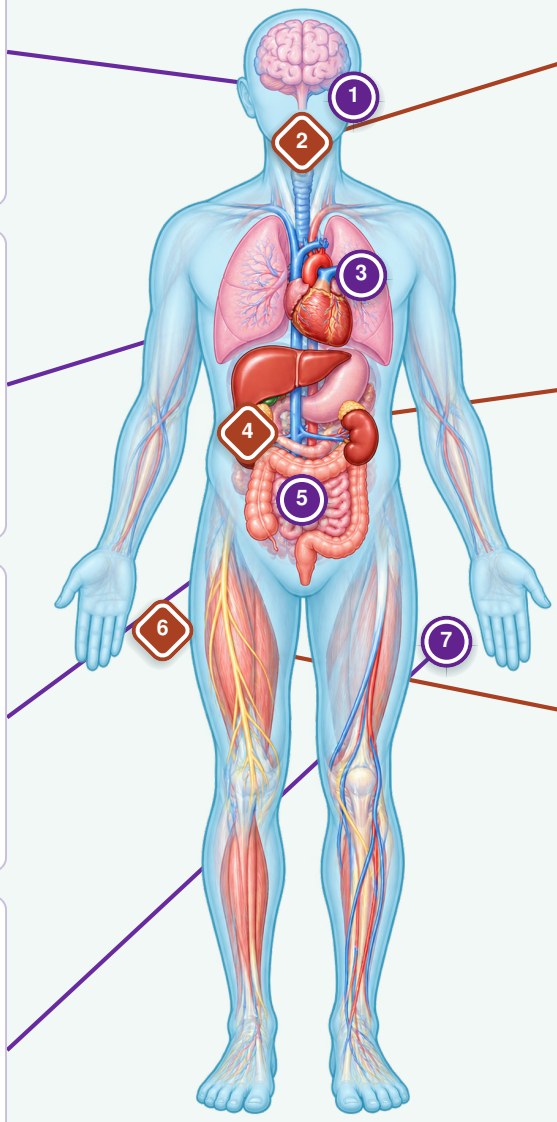
Cisplatin

Report tinnitus or hearing change promptly; discuss existing problems before starting.

2 Mouth

Etoposide

Mouth sores can affect eating or drinking. Seek advice early.



3 Chest / breathing

Both medicines

New breathlessness, wheeze or chest pain needs urgent assessment.

4 Kidneys / fluids

Cisplatin

Report reduced urine or inability to drink; kidney tests guide treatment.

5 Stomach / bowel

Both medicines

Use prescribed anti-sickness medicines; severe vomiting or diarrhoea needs advice.

6 Hands / feet

Cisplatin

Report numbness, tingling or problems with buttons, balance or walking.

7 Skin / blood counts

Both medicines

Report rash, unusual bruising or bleeding. Fever or sudden illness needs urgent advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Recording pages are reusable and do not set course length. The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response. Ask for extra pages when needed. Record hair loss and other changes in the free-text notes.

Etoposide · Topoisomerase II chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Cisplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Urinary symptoms	□ Usual pattern	□ Mild change: report	□ Pain or frequent urination: call	□ Cannot pass urine / marked bleeding: urgent review	□ Collapse or severe illness: 112/999
Hearing changes	□ No change	□ New ringing or muffled hearing: report promptly	□ Conversation difficult: contact the team	□ Sudden hearing loss: urgent assessment	□ Sudden weakness or collapse: 112/999
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

Cisplatin + Etoposide

NCCP 00280 v7 · 21-day cycle

Cisplatin is planned on Day 1. Etoposide is planned on Days 1–3: usually all three days by infusion, or a prescribed alternative with an infusion on Day 1 and capsules twice daily on Days 2–3. The source plans 4–6 cycles, usually every 21 days. The team may delay, reduce or stop treatment according to your health and response.

1	2	3	4	5	6

Your contact plan

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
Emergency: 112 / 999.

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00280/ · no patient information.

Draft for review · SACTCheck v0.84.28

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00280 v7 · official regimen**
   https://healthservice.hse.ie/documents/6927/280_CISplatin_and_Etoposide.pdf
2. **NCCP NCIS Medical Oncology antiemetics V8, Table 1 and multi-day guidance ↗**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **Manufacturer cisplatin product information**
   https://www.medicines.org.uk/emc/product/6960/smpc
4. **Manufacturer etoposide product information: mechanism, risks and precautions**
   https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/United_Kingdom/Etoposide-spc-gb.pdf
5. **HSE chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
6. **COCIS: individual-patient meta-analysis of first-line platinum chemotherapy in small-cell lung cancer (SCLC)**
   https://pubmed.ncbi.nlm.nih.gov/22473169/
7. **JCOG9702: carboplatin/etoposide versus split cisplatin/etoposide in elderly or poor-risk extensive SCLC**
   https://www.nature.com/articles/6603810
8. **HPRA Vepesid capsules: patient leaflet**
   https://assets.hpra.ie/products/Human/30581/94054f4b-91cf-4c49-8063-040ead5c4a92.pdf